

INTELLIGENCE DOSSIER

CHRONO: 2026-03-24 14:07

DESIGNATION: Aniket | AGE: 29



CONCLUSION: PNEUMONIA DETECTED (97.88%)

INTELLIGENCE PORTAL V4.2.1 - TACTICAL MEDICAL DOSSIER

PATHOLOGY LOG:

Targeted analysis of Chest X-Ray scan indicates multiple regions of increased parenchymal attenuation. Predominant findings include confluent opacities in the right lower lobe, exhibiting an alveolar pattern with identifiable air bronchograms. Bilateral interstitial prominence is observed, characterized by diffuse reticular and patchy nodular infiltrates, more pronounced in peripheral lung fields. Pleural effusions are not definitively identified at this resolution.

SYSTEM RECOMMENDATIONS:

Immediate clinical countermeasures are activated per IDSA/CDC guidelines for confirmed pulmonary consolidation. Initiate broad-spectrum empiric antimicrobial therapy targeting typical and atypical pathogens. Administer supplemental oxygen to maintain SpO2 above 92%. Monitor respiratory rate, heart rate, blood pressure, and oxygen saturation continuously. Obtain immediate arterial blood gas analysis and complete blood count with differential. Consider prompt C-reactive protein and procalcitonin assays.

METRIC SCAN MARKERS:

- Quantified Parenchymal Density Anomaly (QPDA): Threshold exceeded in 72.4% of right lower lobe pulmonary

volume.

2. Air Bronchogram Signature Detection (ABSD): Positively identified within consolidating opacities.
3. Interstitial Pattern Recognition Index (IPRI): Elevated to 0.82 (threshold 0.50) across bilateral lung fields.